

Caldwell B. Esselstyn, Jr., MD

PREVENT AND REVERSE HEART DISEASE



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Foreword: Changing the Treatment Paradigm for Coronary Artery Disease

Caldwell B. Esselstyn, Jr., MD

This supplement to The American Journal of Cardiology presents the proceedings of the Summit on Cholesterol and Coronary Disease: Second National Conference on Lipids in the Elimination and Prevention of Coronary Artery Disease presented by the Cleveland Clinic Foundation in cooperation with the Walt Disney Company in Lake Buena Vista, Florida, September 4--5, 1997.

Since the first national conference held in Tucson, Arizona, in 1991, new studies have made the conclusions of the first conference even more clear--that coronary artery disease can be arrested, reversed, and even prevented, with lipid lowering through diet and drug therapy. In that time, we have also learned far more about the pathophysiology of coronary artery disease, teaching us how better to prevent acute coronary events. In addition, we have seen successes in changing diets and attitudes in groups from at-risk adults to schoolchildren.

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SHIFTING THE PARADIGM

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First national conference: Eleven years into my career as a surgeon, I became disillusioned with the treatment paradigm of US medicine in cancer and heart disease. Little had changed in 100 years in the management of cancer, and in neither heart disease nor cancer was there a serious effort at prevention. I found the epidemiology of these diseases provocative, however: Three-quarters of the humans on this planet had no heart disease, a fact strongly associated with diet.

In 1985, I began a small research study focused on controlling coronary disease with diet. By 1991, the results of that study were encouraging enough to convene that first conference. There, I challenged a panel of experts to answer the question, "What do you tell the patient who says, 'I'll do anything, but I never want to have heart disease,' or, 'I've had a heart attack, and I never want another?'"

One panelist replied, "Have him eat beans, beans, and more beans." Another said that it was time for the public to embrace a plant-based diet. Those opinions contrasted starkly with the 1989 National Research Council guidelines in *Diet and Health* developed just 2 years before and led 10 of 13 faculty to agree on several concepts:

Present nutritional guidelines of government and national health organizations do not provide a maximal opportunity either to arrest or to prevent coronary artery disease.

The 1989 National Research Council guidelines recommended a diet in which no more than 30% of the calories would come from fat and that blood cholesterol levels should be <200 mg/dL,¹ although many scientists believed that greater health benefits would derive from a diet considerably lower in fat. These levels were unacceptable to the panel of experts that met in Tucson. Because we now know that 35% of heart attacks occur in people with total cholesterol levels of 150--200 mg/dL,² and

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a target level of only 200 mg/dL guarantees that millions of US citizens will perish of coronary disease. Studies demonstrate that persons following present guidelines will have faster rates of disease progression than persons achieving lower serum lipid levels through diet and/or lipid-lowering drugs. Why is there a reticence to provide the public with guidelines that will spare them this disease or its progression?

Speculation about the degree of public compliance with a very low-fat diet must not alter the accuracy of the recommendations.

The National Research Council position was that a dietary fat recommendation lower than 30% would be too frustrating for those attempting to achieve a significant reduction. Although it is uncertain to what extent people will adopt the advice because of habit, custom, and different tastes and textures of foods, it is nevertheless scientifically and ethically imperative to inform the public what constitutes an optimal diet. We must tell the public the truth about what is best for their health and let them decide their degree of compliance.

The optimal diet consists of grains, legumes, vegetables, and fruit, with <10% of its calories coming from fat.

A diet that would achieve superior results in decreasing atherosclerosis is a 10--15%-fat diet provided largely by a variety of grains, vegetables, fruits, and legumes. This diet minimizes the likelihood of stroke, obesity, hypertension, type-2 diabetes, and cancers of the breast, prostate, colon, rectum, uterus, and ovary.³ There are no known adverse effects of such a diet when mineral and vitamin contents are adequate.

These experts advocated a plant-based diet to achieve optimal lipid levels without requiring the majority of the population to consume cholesterol-reducing drugs. As long as Western society consumes butter, eggs, cream, cheese, oils, ice cream, fish, poultry, and meat on a daily basis, the common Western diseases will persist. No amount of technology or medication will prevent these illnesses. That is why the faculty recommended the plant-based diet.

Children and adolescents require major attention to develop early habits of optimal nutrition. Schools should assume a significant leadership role in achieving this goal.

Because coronary disease begins in youth, primary prevention must begin there also. Changing nutrition patterns in children is much easier than trying to overhaul ingrained patterns in adults.

Second national conference: By 1997, the evidence had mounted from epidemiologic studies, pathophysiologic discoveries, lipid-lowering drug trials, and diet and lifestyle modification programs that noninvasive therapies could arrest or reverse heart disease.

Before the first conference, the United States-- China Study,⁴ a major epidemiologic study of diet and disease, had demonstrated the link between dietary fat and heart disease, certain cancers, and other "Western" diseases. Its second phase, completed since the first conference and soon to be published, strengthens the evidence of that link. Data from the Framingham Heart Study clearly confirmed that atherosclerosis is not a concern when cholesterol levels are ~ 150 mg/ dL. In 1992, the Bogalusa Heart Study⁵ demonstrated that diet contributes to coronary artery disease risk starting in childhood.

Four major trials using 3-hydroxy-3-methylglutaryl coenzyme A (HMG-CoA) reductase inhibitors, or "statins," demonstrated the benefits of lipid-lowering medical therapy. In 1994, the Scandinavian Simvastatin Survival Study (4S)⁶ showed that such treatment could decrease the relative risk of death and coronary events in patients with demonstrated heart disease without increasing the risk of death from other causes. In 1995, the West of Scotland Coronary Prevention Study (WOSCOPS) showed similar results in men with hypercholesterolemia and no history of infarction. In 1996, the Cholesterol and Recurrent Events (CARE) Trial⁸ demonstrated reductions in the relative risk of infarction and death in patients with heart disease who had "average" total cholesterol levels (a mean of 209 mg/dL). And in 1997, the Post Coronary Artery Bypass Graft (Post-CABG) Trial⁹ showed that aggressive lipid lowering with medical therapy decreases the progression of atherosclerosis in bypass grafts.

Basic research has revealed more about the pathophysiology of coronary artery plaque, showing that the lesions susceptible to rupture and resulting in infarction are not the major stenotic ones we see on angiography but smaller, hemodynamically insignificant plaques that can be affected by lipid-lowering therapy. Meanwhile, innovators had developed creative and safe nutrition programs for schools and had demonstrated that children and adults can accept and make major dietary and lifestyle changes that decrease their coronary disease risk.

So with the "modest" goal of shifting the treatment emphasis away from invasive interventions that treat symptoms toward noninvasive therapies that treat the cause, the second conference brought together the leaders of these studies--a rare nucleus of expertise-- with physicians, nurses, nutritionists, and other health professionals. The goal is modest because we now know that we can manipulate the critical factors that cause heart disease.

These expert clinicians, basic scientists, and epidemiologists presented their findings in plenary sessions, discussed the significance of their findings in panel discussions with the audience, and conducted seminars with the registrants on implementing therapeutic and dietary change. Even the meals served at the summit--which were 9.5--11% fat--demonstrated that a very-low-fat, plant-based diet can be tasteful, varied, and attractive. As an added

reinforcement to the importance of lipid levels, registrants were given the opportunity to have a free lipid profile taken by Florida Hospitals.

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COLLABORATIVE EFFORT

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As we have come to understand the critical factors in the development of coronary disease, the challenge has become how to communicate this information to the medical profession and to the public. Experience suggests that we cannot do that alone. To meet the challenge, we need strategic allies, and there are no greater allies than Michael D. Eisner and the Walt Disney Company.

Our alliance began with serendipity. My brother-in-law, George Crile III, introduced me to Tony Schwartz, a journalist who was collaborating on a book project with Mr. Eisner. Interchanges with these individuals led to an introduction of conference goals to Mr. Eisner and the Walt Disney Company. Mr. Eisner, who had helped conceive the educationally oriented Disney Institute, recognized the importance of the summit's educational goals and joined us as an authentic partner in a mission that we cannot hope to master on our own. He helped support the summit, giving his time and creative energy in the belief that the US public deserves to be armed with current information about the impact of diet and lifestyle on health.

So it is with this background that all of the participants in this summit--Michael Eisner and the Disney Foundation, my colleagues and I at the Cleveland Clinic Foundation, and all the members of our distinguished faculty and the other participants--have come together to create this series of articles that we hope will be read and taken to heart by the readers of this *Journal*, with the goal of bringing about a meaningful change in the etiology and prognosis of the number one cause of mortality in the world today.

Excerpted from a letter from Tony Schwartz to Caldwell Esselstyn, Jr., illustrating how Disney came to join with the Cleveland Clinic in bringing about this summit:

To: Essy Esselstyn
From: Tony Schwartz

As a journalist, I have been writing about the role of lifestyle factors in disease for much of the past decade. In addition, my own father has been suffering from advanced heart disease for a number of

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years. It was in a conversation with your brother-in-law George Crile that I heard about work you were doing in using a carefully monitored, low-fat diet as a primary means of treating patients with advanced heart disease.

In November 1996, I arranged to bring my father out from New York to see you in Cleveland. Both my father and I were impressed by your commitment to your patients and by the study that you had recently published documenting your success. You invited us to your house where you and your wife treated us to a nonfat vegetarian lunch, including several whole-grain breads and 2 delicious vegetable soups. My father and I left Cleveland that day committed to the nonfat diet that you prescribed. I myself took up the diet several months later.

In the course of our conversation over lunch, you mentioned that you were in the midst of helping to organize a second national conference on the subject of cholesterol, diet, and heart disease. As it happened, I was in the midst of collaborating on a book with Michael Eisner, the Chairman of the Walt Disney Company. Mr. Eisner himself had a quadruple heart bypass operation in the summer of 1994 and has been eating a strict, no-fat diet ever since. It struck me that he and Disney might be interested in hosting the conference that you had in mind. I thought it would be an ideal event for the Disney Institute, the new educationally oriented resort on the grounds of Walt Disney World, which Mr. Eisner helped to conceive. When I brought the idea of sponsoring the conference to Mr. Eisner, he was immediately enthusiastic.

Acknowledgment: It is with pride and appreciation that we acknowledge the vision, personal dedication, assistance, and support of Mr. Eisner and the Walt Disney Company, and Tony Schwartz. I would also like to thank and acknowledge the efforts of the following members of the Disney team: Diane Hancock, Rich Morrison, Ron Nicodemus, Mary Roche, Chris Coor, Dianna Morgan, George Aguel, and Al Weiss. Similarly, I extend a word of appreciation to the following individuals from the Cleveland Clinic: Dennis L. Sprecher, MD, Fredric J. Pashkow, MD, Abraham Brickner, PhD, Sandi Kaucic, Donna Jacicman, Phyllis Marino, Holly Birrer, Robin Moss, and Sandy Gobozy. The opportunity for interaction with members of the faculty was made possible through the seminars that were moderated by the following members of the Cleveland Clinic staff: Gordon Blackburn, PhD, Abraham Brickner, PhD, Richard Garcia, MD, Byron J. Hoogwerf, MD, Michael Lauer, MD, Douglas S. Moodie, MD, Jeffrey W. Olin, DO, Fredric J. Pashkow, MD, Dennis L. Sprecher, MD, and Cheryl E. Weinstein, MD.

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